

CLINICAL TRIAL

Study of Antithrombotic Treatment After Intracerebral Haemorrhage-Antiplatelets: A Randomized Trial

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BACKGROUND: Patients surviving spontaneous intracerebral hemorrhage (ICH) who have indications for antithrombotic treatment face increased risks of both hemorrhagic and ischemic events. The aim of STATICH (Study of Antithrombotic Treatment After Intracerebral Haemorrhage) was to assess the safety and efficacy of long-term antithrombotic treatment after ICH.

METHODS: STATICH Antiplatelets (EudraCT 2014-002636-13; Unique identifier: NCT03186729) was a randomized, multicenter, open, blinded end point parallel-group trial, including adults with an indication for antiplatelet treatment to either start or avoid antiplatelet treatment. The target sample size was 500 participants. Patients were allocated 1:1 by a Web-based randomization system and followed for a minimum of 2 years. Outcome assessments were done by personnel blinded to treatment. Participants were not blinded. The primary safety outcome was recurrent symptomatic ICH.

RESULTS: The trial was stopped early due to slow recruitment. Sixty-nine patients were included between August 2018 and December 2022: 34 were randomized to start antiplatelet treatment, and 35 were randomized to avoid antiplatelet treatment. No patients were excluded from analyses. The median age was 75 (Q1–Q3, 66–80) years, and 43 (75%) had previous ischemic stroke or transient ischemic attack. Five patients in the start antiplatelet treatment group (15% [95% CI, 6%–31%]), and 1 in the avoid group (3% [95% CI, 0%–18%]) experienced recurrent ICH, while 3 (9% [95% CI, 3%–24%]) and 7 (20% [95% CI, 10%–37%]), respectively, experienced a major ischemic event. A total of 88 serious adverse events and no suspected unexpected serious adverse reactions were registered.

CONCLUSIONS: There were numerically more recurrent ICHs and fewer ischemic events in patients starting antiplatelet treatment after ICH compared with those avoiding antiplatelet treatment. The slow recruitment might be due to clinicians being certain of the best treatment strategy or patients being too frail to participate. STATICH will contribute to a planned collaborative individual patient-data meta-analysis to resolve this dilemma.

GRAPHIC ABSTRACT: A [graphic abstract](#) is available for this article.

Key Words: cerebral hemorrhage ■ ischemic stroke ■ platelet aggregation inhibitors ■ randomized controlled trial ■ secondary prevention

Observational studies show that ischemic major adverse cardiovascular events are more frequent in survivors of spontaneous intracerebral hemorrhage (ICH) compared with the general population,^{1,2}

and ischemic events are more frequent than recurrent ICH.^{3,4}

Data from a Norwegian population suggest that about 25% use antiplatelet treatment at the time of ICH,⁵ but

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Nonstandard Abbreviations and Acronyms

CT	computed tomography
ICH	intracerebral hemorrhage
MRI	magnetic resonance imaging
mRS	modified Rankin Scale
STATICH	Study of Antithrombotic Treatment After Intracerebral Haemorrhage

the rate varies between studies from 4% to 40%.^{6–8} Thus, adults with a history of vascular disease who suffer a stroke due to ICH are faced with the dilemma of starting versus avoiding antiplatelet treatment.

To date, only 1 randomized trial addressing the question of whether to (re)start or avoid antiplatelet treatment following ICH has been published.⁹ RESTART (Restart or Stop Antithrombotics Randomized Trial) included 537 patients who were followed up for a median time of 2 years and found no significant difference in risk of recurrent ICH but a tendency toward fewer ICH and ischemic strokes in the group who received antiplatelet treatment. Uncertainty remains about what is the best treatment option for adults with ICH and concomitant antithrombotic treatment.

The STATICH (Study of Antithrombotic Treatment After Intracerebral Haemorrhage) aimed to investigate the safety and efficacy of long-term antithrombotic treatment after stroke due to ICH in patients with vascular disease.

METHODS

Data Availability

The data sets used are available from the corresponding author upon reasonable request.

Trial Design

STATICH (EudraCT 2014-002636-13; Unique identifier: NCT03186729, registered June 14, 2017) was a Scandinavian investigator-initiated, prospectively randomized, open-blinded end point trial of parallel groups with a 1:1 allocation. The trial was conducted in accordance with the principles of good clinical practice and the Declaration of Helsinki.¹⁰ Ethical approval was obtained in each country before the start of the trial. The trial included patients in 2 separate trials: one antiplatelet trial (STATICH Antiplatelets) and one anticoagulant trial (STATICH Anticoagulants; Figure 1). We will here report the results of the STATICH Antiplatelets trial. This article follows the CONSORT reporting guidelines (Consolidated Standards of Reporting Trials).¹¹ The results of STATICH Anticoagulants will be published elsewhere.

Changes to Trial Protocol

The full protocol of STATICH has been published previously.¹² The protocol was amended after the initiation of the trial: cognitive assessment was dismissed due to the pragmatic nature of the trial and cognitive assessment not being part of routine clinical practice, the maximum time from ICH to inclusion was changed from 180 days to no upper limit, and the inclusion period was extended. The latter 2 amendments were made to increase the number of included patients. The latest version of the protocol and the statistical analysis plan were

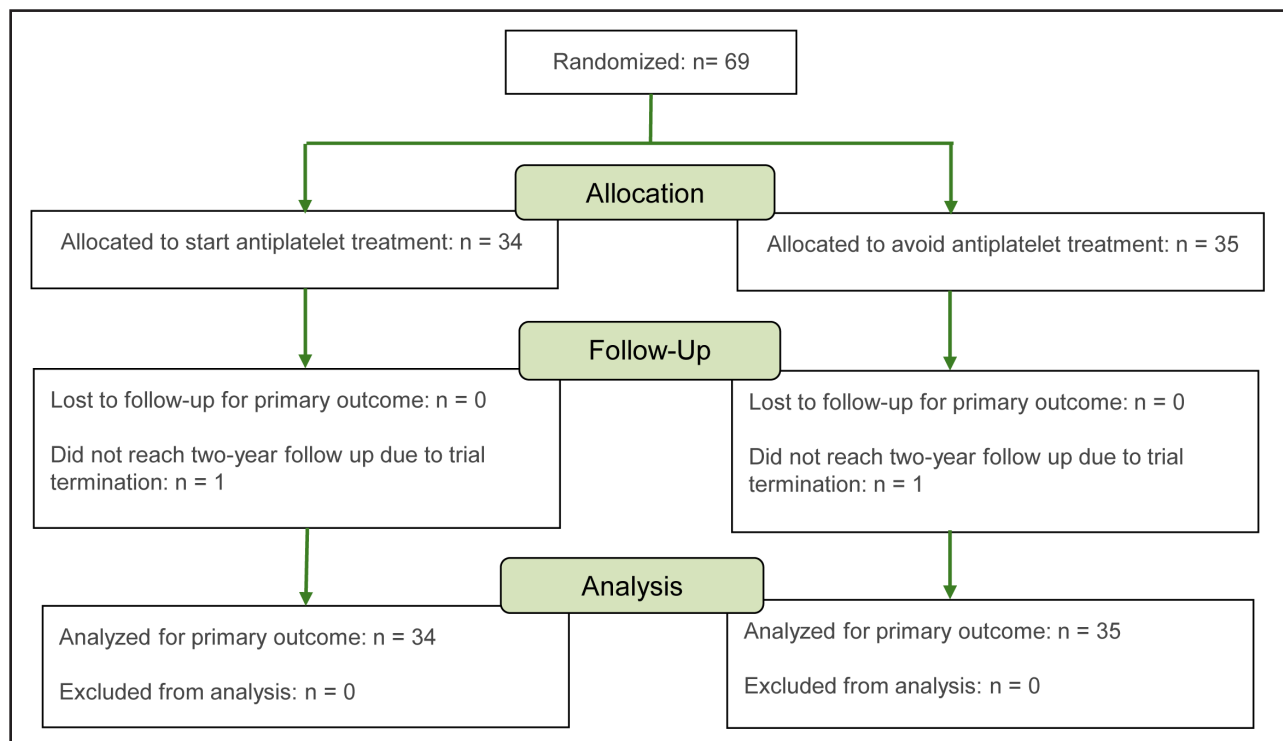


Figure 1. Trial profile STATICH (Study of Antithrombotic Treatment After Intracerebral Haemorrhage) antiplatelets.

approved by the Trial Steering Committee and are included in the [Supplemental Material](#) and available at www.statich.no.

Trial Setting

Patients were enrolled from 25 hospital sites in Norway, Sweden, and Denmark ([Table S1](#)) and could be recruited either during their hospital admission or later in an outpatient clinic.

Data Collection

At randomization, investigators recorded the patient's age, sex, living arrangements, functional status, medical history, symptom onset date, location of index ICH (lobar versus nonlobar), prior use of antithrombotic medication, blood pressure, serum creatinine levels, brain imaging used to identify ICH (computed tomography [CT] and magnetic resonance imaging [MRI]), and the intended antiplatelet treatment recommended by the treating physician if the patient was allocated to start antiplatelet treatment. A brain CT or MRI scan confirming the ICH was obtained before inclusion. The CT/MRI at baseline was assessed by neuroradiologists concerning ICH location and size.

Adherence to treatment allocation was assessed by telephone interview every 6 months, done by trained personnel at the national coordinating trial centers who knew the patient's allocation to treatment. If the patient or carer could not be contacted, we contacted the patient's general practitioner.

Outcomes were collected by other trained personnel at the national coordinating trial centers who were blinded to the treatment allocation. All outcome assessments were done by telephone every 6 months, and reported outcomes were centrally confirmed by collecting patient records from hospitals or general practitioners.

Patients were followed for at least 2 years after inclusion (or death), except for 2 patients who did not reach 2-year follow-up due to withdrawal from the trial. All patients were included in the analyses.

Eligibility Criteria

Patients were eligible for inclusion if they were ≥ 18 years of age, had experienced a nontraumatic ICH with no underlying structural cause at least 24 hours before inclusion, and had an indication for antiplatelet treatment. Indications for antiplatelet drugs could be secondary prevention due to previous ischemic vascular disease, arterial stents, or primary prevention if patients had known atherosclerotic arterial disease. All patients performed a mandatory brain CT or MRI scan before inclusion, and all had to give written consent. In Sweden and Denmark, the written consent had to be provided by the patient. In Norway, a relative or representative could give proxy consent if the patient did not have the capacity.

Exclusion criteria comprised a compelling indication for antiplatelet treatment (eg, recent coronary artery stenting), contraindications to the antiplatelet drug (eg, low thrombocyte count), pregnancy, breastfeeding or being of childbearing potential without the use of contraception, and malignancy with a life expectancy of < 2 years. See [Protocol](#) in the [Supplemental Material](#) for further details on trial eligibility.

Intervention and Comparator

Patients were randomized to start antiplatelet treatment or avoid antiplatelet treatment. In the start antiplatelet treatment

group, the choice of antiplatelet drug and dosage was decided by the including physician. The first prescription for the antiplatelet drug was made by the including physician, and later prescriptions were filled by the patient's general practitioner. Dual-antiplatelet treatment was allowed. Antiplatelet agents are prescription-only in Norway and Sweden but can be bought over the counter in Denmark. The use of over-the-counter antiplatelet agents was not specified in the protocol. All use of antiplatelet treatment was assessed during open follow-up, regardless of prescription or over-the-counter use. In the avoid antiplatelet treatment group, patients were to abstain from all types of antithrombotic treatment. Concomitant treatment followed standard care and was not determined or interfered with by the trial.

Outcomes

The primary safety outcome was the occurrence of a recurrent, symptomatic ICH. The definition of ICH was bleeding within the brain parenchyma or into the ventricular system, that is, intracranial bleedings that were not extradural, subdural, or subarachnoid. All initial scans were collected and reviewed by neuroradiologists blinded to treatment allocation. All primary outcomes reported by patients were confirmed by obtaining medical records from hospitals and by adjudication from neuroradiologists interpreting brain CT or MRI scans of the primary outcome.

Secondary outcomes were prespecified in the protocol as vascular death, death of any cause, symptomatic epidural, subdural or subarachnoid hemorrhage, symptomatic major extracranial hemorrhage, clinically relevant nonmajor bleeding, ischemic events, and functional outcome at 2 years according to the modified Rankin Scale (mRS) score. Due to the small sample size, the Trial Steering Committee decided to change secondary outcomes to composite outcomes before finalizing the statistical analysis plan. Prespecified secondary outcomes according to the statistical analysis plan were major ischemic events (a composite end point of ischemic stroke, myocardial infarction, mesenteric ischemia, peripheral artery occlusion, deep vein thrombosis, pulmonary embolism, and revascularization procedures of carotid, coronary, or peripheral arteries), major hemorrhagic events (a composite end point of symptomatic ICH), other symptomatic intracranial hemorrhages (epidural, subdural, or subarachnoid, either spontaneous or traumatic), symptomatic major extracranial hemorrhage requiring transfusion with > 2 units of whole blood or red cells, a fall in > 2.0 -g/L hemoglobin, bleeding in a critical area or organ or fatal hemorrhage, major adverse cardiovascular events (major adverse cardiovascular events, a composite end point of ischemic stroke, myocardial infarction, other major ischemic events, ICH, other intracranial hemorrhage, major extracranial hemorrhage, or vascular death), death of all causes, and functional status at 2 years according to mRS score.

Harms

Serious adverse events were assessed systematically biannually by telephone interview and by collecting data from medical records. Serious adverse events were defined in accordance with Guidelines on Good Clinical Practice.¹³ Unexpected serious adverse events were defined as events not prespecified in the protocol and were to be reported by the trial investigator to the

trial administration within 24 hours. In addition, an unexpected serious adverse event form should be filled out, where the seriousness of the event and causality between the intervention and the event should be assessed. All serious adverse events were reported to the national medicinal product authorities.

Sample Size

Sample size calculation was performed before the start of the trial and has not been recalculated throughout the trial. The total sample size was estimated to be 500 patients, assuming a significance level of 5%, a power of 80%, and an increase in the annual rate of recurrent ICH from 2% to 8% with anti-thrombotic treatment. Although STATICH consists of 2 separate trials, STATICH Antiplatelets and STATICH Anticoagulants, the sample size calculation was performed for the STATICH trial as one trial.

Randomization and Allocation Concealment

A secure website was used to randomly assign patients to either start antiplatelet treatment (intervention) or avoid antiplatelet treatment (control), with 1:1 allocation. The random allocation sequence was generated by the trial programmer. A minimization algorithm without a predetermined sequence was used, minimizing the difference between treatment groups with respect to baseline characteristics.¹⁴ The first patient was randomly allocated with a probability of 0.5 to one of the arms. Each subsequent patient was allocated with a probability of 0.8 to the arm that minimized the difference between arms using 5 minimization criteria. The minimization criteria were age (<70 versus ≥70 years), location of index ICH (only supratentorial lobar regions versus other regions affected), probability of a good outcome at 6 months (<0.15 versus ≥0.15),⁹ days from index ICH to randomization (0–6 versus 7–30 versus >30 days), and choice of antiplatelet drug decided by the patient's physician if allocated to antiplatelet treatment (aspirin only versus another antiplatelet drug or combination of antiplatelet drugs).

Blinding

Outcome assessment was performed by study personnel blinded to treatment allocation and patient medications, whereas participants, including physicians and study personnel carrying out adherence assessments, were not blinded.

Statistical Methods

The statistical analyses followed the prespecified statistical analysis plan, which was finalized and published on the trial website and clinicaltrials.gov before database lock ([Supplemental Material](#)). This plan stated that analyses would be performed for the 2 trials separately.

The prespecified analysis for the primary outcome was time-to-first event using Cox proportional hazard regression models. The model was planned to be adjusted for the variable used as the minimization criterion in the randomization system. If adjustment for all these variables was not possible due to few events, age (<70 versus ≥70 years) and ICH location (lobar only versus other location) were to be prioritized. However, due to the small sample size with a limited number of events, such analysis was not feasible. Results are, therefore, reported as

absolute numbers and illustrated with Kaplan-Meier plots without statistical inference. This was stated as an option if there were too few events to perform the planned statistical analyses.

The secondary analyses were prespecified as time-to-event analyses of all secondary outcomes within a 2-year follow-up and all dichotomous outcomes at complete follow-up. As for the primary analysis, there were not enough events, and results are, therefore, presented analogous to the primary outcome.

The functional outcome at 2-year follow-up measured by mRS score had enough events and was, therefore, assessed by planned ordinal logistic regression analysis, adjusting for age <70/≥70 years and lobar hematoma location. For this analysis, an assumption of proportional odds was assessed.

All tests were 2-sided, and the level of significance was a *P* value of <0.05.

Analyses were performed in accordance with the intention-to-treat principle. Patients with missing values on the primary outcome were excluded from the analyses.

The statistical analyses were conducted using STATA software, version 18.¹⁵

RESULTS

From August 31, 2018, to December 23, 2022, a total of 69 patients were included in the STATICH Antiplatelets trial (Figure 1). Due to slow recruitment and a lack of resources, the Trial Steering Committee decided to terminate the trial prematurely. The last follow-up was performed on August 31, 2024, leaving 2 patients (one in each treatment group) with follow-up shorter than 2 years.

The median age was 75 (Q1–Q3, 66–80) years, and 43 (62%) patients were male. The median time from index ICH to randomization was 120 (Q1–Q3, 84–180) days. Ninety percent of patients had known hypertension. Mean blood pressure was 139/82 (SD, 15/11) mmHg at the time of inclusion. Patients had predominantly lobar hematoma location (51 [74%]) according to the including clinician, but, when ICH location was reinvestigated by the trial radiologist, 25 (39%) were found to be lobar ([Table S2](#)). Previous ischemic diseases were the indication in 64 patients (93%). Five participants had used antiplatelet treatment either as primary prevention or had asymptomatic radiological findings of ischemic cerebrovascular disease. Further baseline characteristics are presented in Table 1. The median complete follow-up was 3.3 years, constituting a total of 222 person-years.

Five patients (15% [95% CI, 6%–31%]) in the start antiplatelet treatment group and 1 patient (3% [95% CI, 0%–18%]) in the avoid group experienced a new symptomatic ICH (Table 2). The Kaplan-Meier curve is presented in Figure 2. All patients with recurrent ICH had a previous symptomatic ischemic event, and 5 of the 6 patients had a previous ischemic stroke or transient ischemic attack. Of the recurrent ICH, 3 were lobar (all had a lobar index ICH), 1 was deep, 1 was infratentorial, and 1 had >1 location.

Table 1. Baseline Characteristics STATICH Antiplatelets

	Start antiplatelet treatment (n=34)	Avoid antiplatelet treatment (n=35)
Age, y; median (Q1–Q3)	76 (59–81)	75 (66–79)
Male sex, n (%)	20 (59)	23 (66)
mRS score, median (Q1–Q3)	2 (1–3)	1 (1–3)
Previous ischemic stroke or TIA, n (%)	25 (74)	27 (77)
Previous ischemic heart disease, n (%)	6 (18)	11 (31)
Previous peripheral artery disease, n (%)	1 (3)	2 (6)
Other indication for antiplatelet treatment, n (%)	4 (12)	1 (3)
Hypertension, n (%)	30 (88)	32 (91)
Systolic blood pressure, mm Hg; mean (SD)	143 (13.9)	136 (16.4)
Diastolic blood pressure, mm Hg; mean (SD)	82 (10.4)	82 (10.9)
Diabetes, n (%)	2 (6)	7 (20)
Renal failure, n (%)	4 (12)	1 (3)
Serum creatinine level, $\mu\text{mol/L}$; mean (SD)	83 (36.5)	73 (18.9)
Use of antiplatelet drug before inclusion, n (%)	26 (76)	31 (89)
Lobar ICH according to the clinician, n (%)	26 (76)	25 (71)
Volume of ICH, mL; median (Q1–Q3)	4.6 (0.5–14.0)	7.0 (1.1–13.1)
Days from ICH to inclusion, median (Q1–Q3)	130 (98–186)	115 (75–180)

ICH indicates intracerebral hemorrhage; mRS, modified Rankin Scale; Q1–Q3, range from 25% percentile to 75% percentile; STAT-ICH, Study of Antithrombotic Treatment After Intracerebral Haemorrhage; and TIA, transient ischemic attack.

For the secondary outcomes, major ischemic events occurred in 3 patients (9% [95% CI, 3%–24%]) in the start antiplatelet treatment group and 7 patients in the avoid group (20% [95% CI, 10%–37%]), major hemorrhagic events in 6 patients in the start antiplatelet treatment group (18% [95% CI, 8%–34%]) and 3 patients in the avoid group (9% [95% CI, 3%–24%]), major adverse cardiovascular events in 8 patients in the start antiplatelet treatment group (24% [95% CI, 12%–41%]) versus 9 patients in the avoid group (26% [95% CI, 14%–43%]), and death of all causes in 9 patients in the start antiplatelet treatment group (26% [95% CI, 14%–44%]) and 3 patients in the avoid group (9% [95% CI, 3%–24%]; Table 2). All major hemorrhagic events were intracranial.

For functional status, there was a shift in mRS score in favor of avoiding antiplatelet treatment (adjusted common odds ratio, 2.52 [95% CI, 1.03–6.14]). Post hoc analysis, additionally adjusting for baseline mRS score, found similar results (adjusted common odds ratio, 2.52 [95% CI, 1.01–6.30]).

Of patients allocated to treatment, 20 (59%) used acetylsalicylic acid (Table S3), and none were treated with dual-antiplatelet treatment. Adherence in the start antiplatelet treatment group, measured biannually, varied between 71% and 100% during follow-up and in the avoid antiplatelet group between 67% and 89% (Table S4). In total, 50 patients (85%) were treated with antihypertensive drugs at 2-year follow-up (Table S5).

A total of 88 serious adverse events were registered: 52 in the start antiplatelet treatment group and 36 in the avoid group (Table 2). There were no suspected

unexpected serious adverse reactions. All serious adverse events are listed in Table S6.

DISCUSSION

The STATICH Antiplatelets trial aimed to investigate the safety and efficacy of long-term antiplatelet treatment after spontaneous ICH in patients with vascular disease. Due to slow recruitment and lack of funding, the trial was terminated before reaching the target sample size.

For the primary outcome, there were numerically more recurrent ICHs in the start antiplatelet treatment group than in the avoid group. The secondary analyses of major ischemic events, major hemorrhagic events, and major adverse cardiovascular events suggest that antiplatelet treatment has an anticipated effect of fewer ischemic events and a higher risk of more hemorrhagic events, but these results should also be interpreted with caution. There were numerically more major ischemic events in the avoid antiplatelet treatment group than in the start antiplatelet treatment group (7 versus 3). Due to few events, we could not demonstrate the inferiority of starting antiplatelet treatment.

Functional status at 2 years was in favor of avoiding antiplatelet treatment. This result may be driven by the increased number of recurrent ICH and deaths in the start antiplatelet group compared with the avoid antiplatelet group. This was consistent when performing a post hoc analysis adjusting for baseline mRS score. However, this is a secondary analysis in a trial with low power, and results should be interpreted with caution.

Table 2. Frequencies of First Occurrences of Primary and Secondary (Composite and Individual) Outcomes and Numbers of All Serious Adverse Events and Suspected Unexpected Serious Adverse Reactions Within Complete Follow-Up

	Start antiplatelet treatment (n=35)	Avoid antiplatelet treatment (n=34)
Primary end point		
Recurrent intracerebral hemorrhage	5	1
Secondary end points		
Major ischemic events	3	7
Ischemic stroke	2	7
Myocardial infarction	0	0
Mesenteric ischemia	0	0
Peripheral arterial disease	0	0
Deep vein thrombosis	0	0
Pulmonary embolus	1	0
Coronary artery intervention	0	0
Carotid artery intervention	0	0
Major hemorrhagic events	6	3
Recurrent intracerebral hemorrhage	5	1
Other intracranial hemorrhage	1	2
Major extracranial hemorrhage	0	0
Major adverse cardiovascular events	8	9
Death by any cause	9	3
Other secondary outcomes		
Transient ischemic attack	1	1
Unstable coronary syndrome	0	0
Retinal artery occlusion	0	0
Amaurosis fugax	0	0
Cerebral venous thrombosis	0	0
Clinically relevant nonmajor bleeding	1	1
Serious adverse events	52	36
Suspected unexpected serious adverse reactions	0	0

Only 2 patients were included within the first month after symptom onset. Because the recurrence risk of ICH is particularly elevated in the first 3 months after symptom onset,³ the long inclusion period makes our results even more difficult to interpret.

Control of other risk factors than use of antiplatelet treatment does not always get the same attention in clinical practice, exemplified by stroke patients having less controlled blood pressure than patients with myocardial infarction.¹⁶ Blood pressure in our population was relatively well controlled at the time of randomization; however, we do not have any data on blood pressure levels after inclusion, and beyond that, 50 patients in total (85%) were treated with antihypertensive drugs at 2-year follow-up.

The median ICH volume was 5.8 mL. As in other ICH trials, patients with larger hematoma volumes were underrepresented. A possible reason is that large hematomas are associated with worse functional outcome,¹⁷ which can complicate participation in trials with long-term follow-up. Functional status at baseline in our trial, with a median mRS score of 1, might support this theory.

STATICH Antiplatelets has many limitations, of which small sample size and few events are the most important. In addition, the trial population is selected with an underrepresentation of women and those with larger hematomas. The open-label design introduces the risk of detection bias, and the low recruitment rate introduces selection bias.

The high proportion of lobar ICH may indicate a selection bias because of the greater risk of recurrent ICH in this group. After the end of the trial, we conducted a survey among the principal investigators regarding recruitment considerations ([Supplemental Material](#)). The majority of principal investigators reported that participants were primarily included when treatment uncertainty was high, and uncertainty regarding the use of antiplatelet treatment is greater with lobar ICH and the presence of microbleeds.

The small size of the ICHs and low median mRS score at baseline may also indicate a selection bias. Unfortunately, we do not have complete screening logs. The screening logs from the 3 national coordinating centers

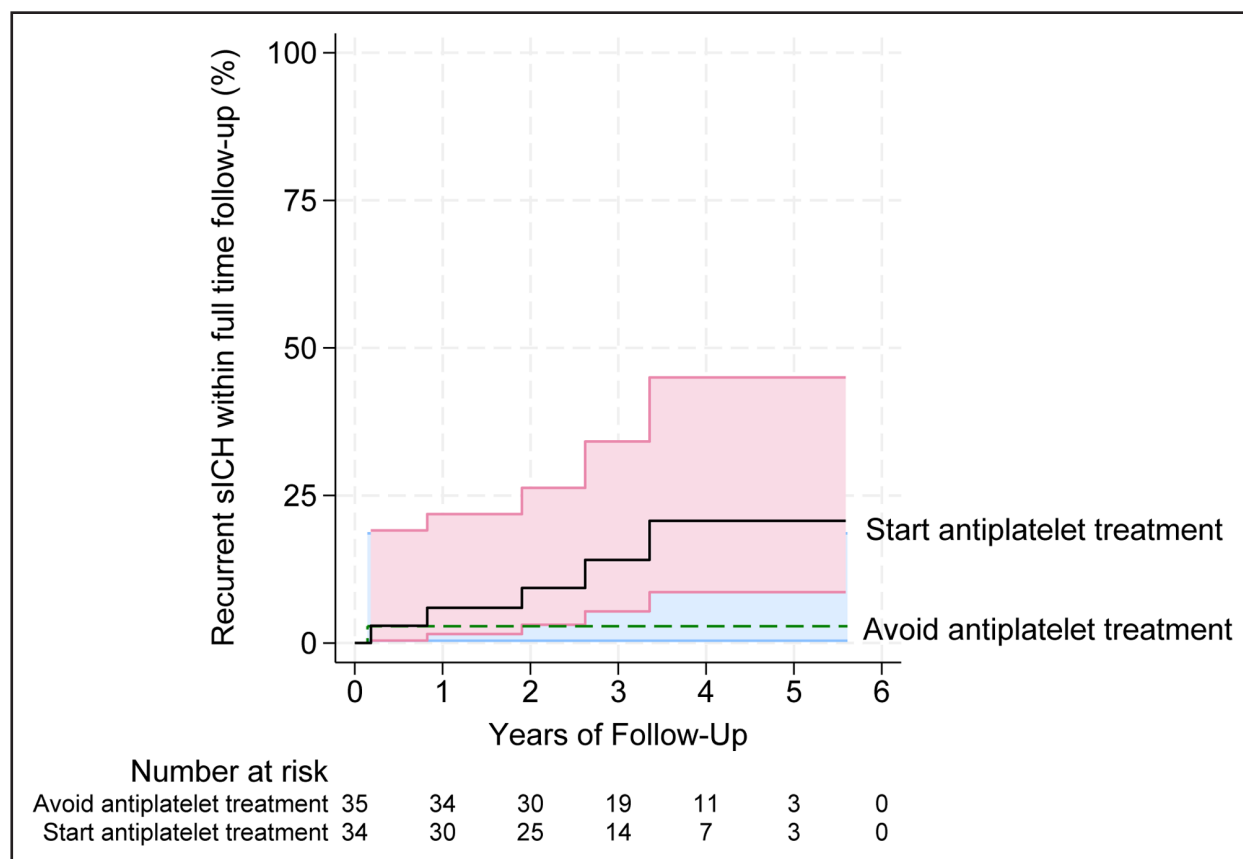


Figure 2. Kaplan-Meier curve of time-to-first recurrent symptomatic intracerebral hemorrhage (sICH).

of patients that fit the inclusion criteria for either STAT-ICH Antiplatelets or STATICH Anticoagulants showed that 45% who met the inclusion criteria had at least 1 exclusion criterion ($n=165/364$) and 38% ($n=137/364$) were considered too unwell either physically or cognitively without meeting exclusion criteria. Of the remaining 62 patients, 38 declined to participate, and 24 were included (Table S7). The information from this limited screening log is consistent with results from the survey, indicating that the main reasons for noninclusion were patients being too frail and poor clinical outcomes after ICH.

We have no additional data on causes of ICH in the included patients but plan to analyze MRI findings from baseline and 2-year follow-up in a planned substudy.

Our trial suffered from slow recruitment (Figure S1). This may in part be due to the trial design, where many small study sites were included instead of fewer and larger sites, creating more complex logistics and not exploiting economies of scale in this investigator-led trial with limited resources. However, we also think that a possible reason might be that clinicians in most cases felt certain about the best approach for secondary prevention in their particular patient, and thus, the ethical assumption for inclusion was not fulfilled.¹⁸ In our opinion, the evidence regarding optimal treatment in patients at high

risk of recurrent ICH, as well as ischemic events, is still insufficient, and definitive evidence is required from the ongoing Antiplatelet Secondary Prevention International Randomised Study After Intracerebral Haemorrhage (www.ASPIRING.ed.ac.uk; ISRCTN16705062) and the planned individual patient-data meta-analysis of similar trials.¹⁹

CONCLUSIONS

STATICH found numerically more recurrent ICH and fewer ischemic events in the group starting antiplatelet treatment. Due to the small sample size and few events, we could not demonstrate the inferiority of starting antiplatelet treatment. Further large trials and meta-analysis are needed to clarify optimal antiplatelet strategies after ICH, and we encourage randomizing patients in the ongoing trials investigating this.²⁰ If this is not an option, we suggest that patients should have a good indication for antiplatelet treatment before (re)starting it after an ICH.

ARTICLE INFORMATION

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Disclosures

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Supplemental Material

Tables S1–S7
Figure S1
Latest Version of Protocol
Statistical Analysis Plan
Survey to Primary Investigators
CONSORT 2025 Checklist

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